3.10 Hemothorax*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel dizzy and short of breath." **Objective** • Blood in the chest drain • BP 92/58 mmHg, HR 118/min • Pale, cold skin	**ACTUAL** Inadequate fluid volume related to blood loss into the pleural space as evidenced by hypotension, tachycardia and sanguineous chest drainage.	• Vital signs stable and drainage falling within 12 hours.	• Take vital signs every 15–30 minutes until stable. • Measure drain output hourly. • Report drainage above the surgeon's limit, often more than 200 mL/hour. • Give IV fluids and blood products as prescribed. • Follow the hemoglobin.	• Outcome reached when vital signs settle and drainage decreases.
Subjective • "I cannot breathe deeply." **Objective** • Breath sounds reduced on the affected side • SpO_2 90%	**ACTUAL** Impaired gas exchange related to lung compression by blood as evidenced by hypoxemia and reduced breath sounds.	• SpO_2 94% or above.	• Give oxygen as prescribed. • Sit him semi-Fowler. • Keep the chest drain patent.	• Considered met at target SpO_2.
Subjective *No specific complaint* **Objective** • Bleeding continues	**RISK** Risk for shock as evidenced by risk factors: hypovolemia from ongoing blood loss.	• Perfusion adequate: MAP above 65 mmHg, urine at least 0.5 mL/kg/hour.	• Insert two large-bore cannulas as ordered. • Measure urine output hourly. • Prepare for thoracotomy if bleeding does not stop.	• Goal reached while perfusion stays adequate.

* **References for this care plan:** [2], [35], [37], [128]. Full entries are listed in the References section of the CPMN book.